EARNINGS UPDATE

Eli Lilly and Company

NYSE: LLY · Q2 2026 (quarter ended 30 June 2026) · Reported 5 August 2026 · Pharmaceuticals / Large-cap biopharma

A beat-and-raise quarter of unusual size, wrapped around a GAAP result that looks worse than the business performed. Revenue of $22.97bn grew 48% year on year and cleared consensus by roughly 11%. Non-GAAP EPS of $8.38 absorbed $3.03 per share of acquired in-process R&D from four completed acquisitions; excluding that charge the underlying figure was $11.41. Full-year revenue guidance went up $2.5bn at the midpoint and the performance-margin range rose two points. The central tension in the quarter is not demand — it is price.

Key takeaways

- Revenue $22.97bn, +48% YoY, ahead of the ~$20.73bn consensus by 10.8%. Growth was 60 points of volume against a 13-point price decline.
- Mounjaro ($9.94bn, +91%) and Zepbound ($4.93bn, +46%) contributed $6.29bn of the $7.42bn absolute revenue increase — 85% of all growth.
- International revenue grew 80% and is now 37% of the total, but at a 36% lower realised price. The mix shift is structurally dilutive to price even as it adds volume.
- FY2026 revenue guidance raised to $85–87bn from $82–85bn; performance margin raised to 49.0–50.5% from 47.0–48.5%; non-GAAP EPS narrowed to $35.50–36.50 from $35.50–37.00 as a $2.78 underlying raise was more than offset by $3.03 of Q2 IPR&D.
- $13.3bn of cash went out the door on acquisitions and IPR&D in the first half, against $16.0bn of operating cash flow. Lilly is converting an earnings windfall into pipeline.
- Orforglipron (Foundayo) is early and slower than hoped; management pointed to a late-July inflection. Retatrutide Phase 3 is complete with a US BLA targeted for Q1 2027, but the biologic-versus-small-molecule filing pathway is unresolved and in litigation.

Results snapshot

Metric	Q2 2026	Q2 2025	Change
Revenue ($m)	**22,974**	**15,558**	**+47.7%**
Gross margin, non-GAAP ($m)	19,831	13,232	+49.9%
Gross margin %, non-GAAP	86.3%	85.0%	+130 bps
R&D ($m)	3,819	3,336	+14.5%
Marketing, selling & admin ($m)	3,430	2,753	+24.6%
Acquired IPR&D ($m)	2,776	154	n.m.
Operating income, GAAP ($m)	8,978	6,867	+30.7%
Performance margin	54.8%	45.8%	+900 bps
Effective tax rate, non-GAAP	22.2%	16.5%	+570 bps
Diluted EPS, GAAP ($)	7.94	6.29	+26.2%
Diluted EPS, non-GAAP ($)	**8.38**	**6.31**	**+32.8%**
Diluted EPS, non-GAAP ex-IPR&D ($)	**11.41**	**6.31**	**+80.8%**

Source: Eli Lilly Form 8-K furnished 5 August 2026 and Form 10-Q for the quarter ended 30 June 2026. Performance margin is a company-defined measure: gross margin less R&D and MS&A, divided by revenue.

1. The quarter in detail

Revenue

Worldwide revenue of $22,974m rose 47.7% against $15,558m a year ago. On a six-month basis revenue of $42,773m is up 51.2% on $28,286m. The absolute increase in the quarter was $7,416m — larger than the entire quarterly revenue of most pharmaceutical companies.

The composition of that increase is highly concentrated. Mounjaro added $4,744m and Zepbound added $1,547m. Together the two tirzepatide brands supplied $6,291m, or 84.8%, of all incremental revenue. Jardiance contributed a further $542m, helped by a comparison distorted by the timing of Boehringer Ingelheim alliance revenue. Strip out cardiometabolic health entirely and the remaining $4.6bn of the business grew 9.7% — respectable, but a different company from the one the headline describes.

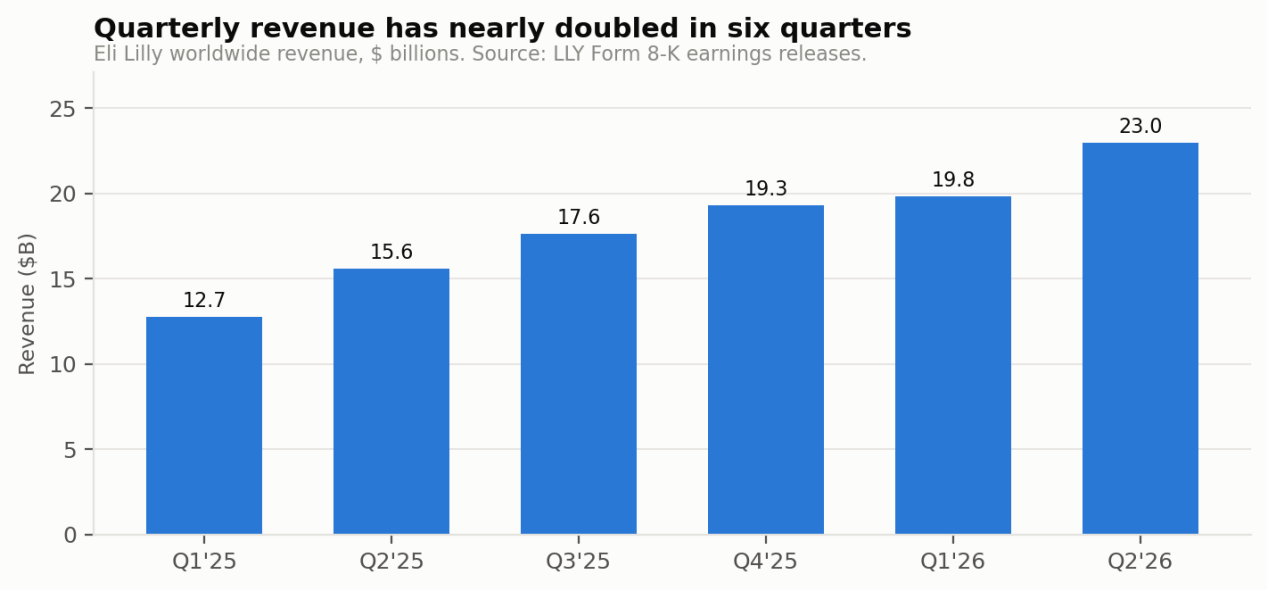


Figure 1 — Quarterly revenue, Q1 2025 to Q2 2026. Q3 2025 is derived as FY2025 less the three reported quarters.

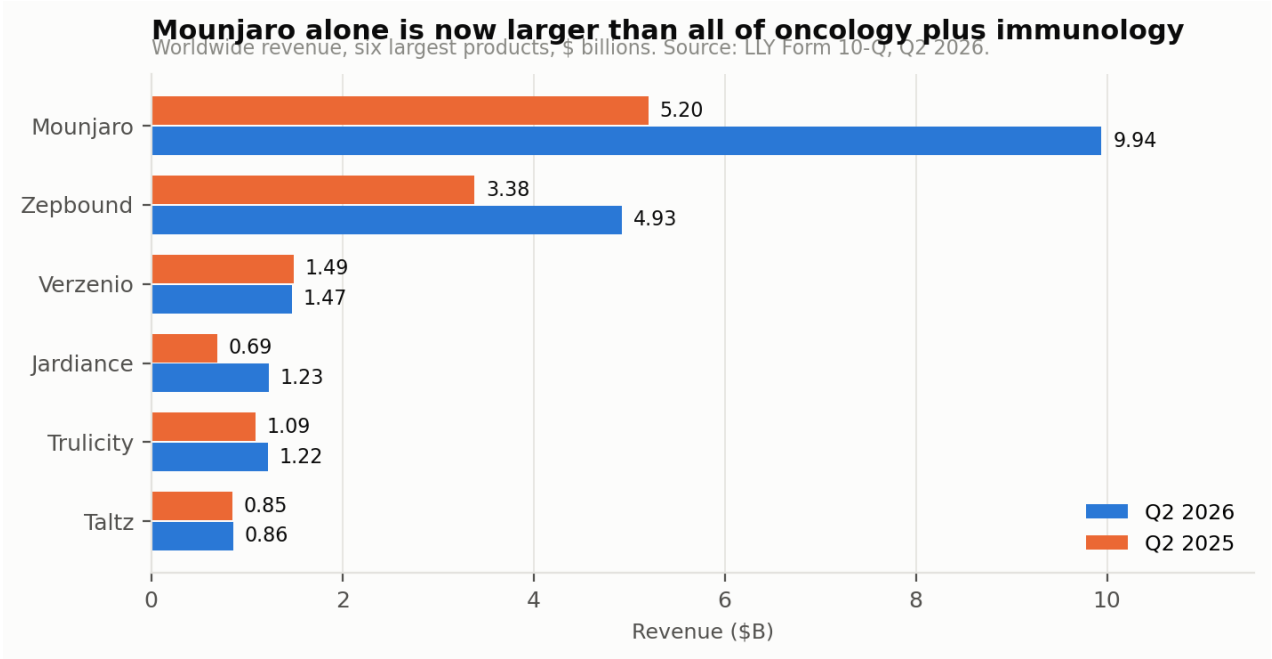


Figure 2 — Six largest products, Q2 2026 versus Q2 2025, worldwide revenue.

Therapeutic areas

Cardiometabolic health revenue of $18,353m grew 61.8% and now represents 79.9% of the company, up from 72.9% a year ago. Oncology grew 6.5% to $2,570m, held back by Verzenio, which declined 1% to $1,474m as US revenue fell 9% — the first sign of erosion in a franchise facing an eventual loss of exclusivity. Immunology grew 12.8% to $1,417m with Taltz essentially flat and the growth coming from Ebglyss and Omvoh. Neuroscience grew 24.7% to $429m, driven by Kisunla.

Management framed oncology, immunology and neuroscience as having grown "121% versus prior year" collectively. That figure refers to the newer products within those areas rather than the areas themselves, which grew 9.6% in aggregate. Both statements are true; only one describes the P&L.

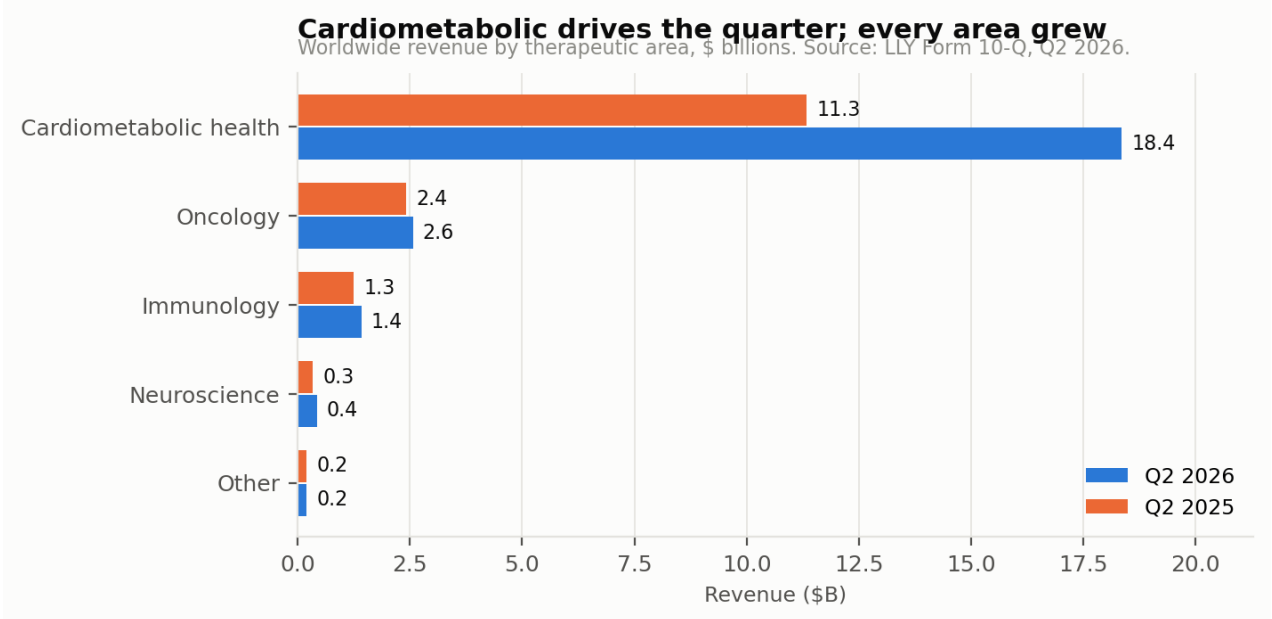


Figure 3 — Revenue by therapeutic area, Q2 2026 versus Q2 2025.

Geography and the price problem

US revenue of $14,413m grew 33%, comprising a 37% volume increase against a 3% decline in realised prices. Revenue outside the US of $8,561m grew 80%, comprising a 113% volume increase against a 36% decline in realised prices. Worldwide, 60 points of volume growth were partially offset by 13 points of price decline.

The reported 3% US price decline flatters the underlying trend. CFO Lucas Montarce disclosed on the call that the quarter benefited from one-time revisions to rebate estimates; excluding those, US price declined 9%, driven by Zepbound and Mounjaro. That is the number to carry forward. Management was explicit that the one-time benefit does not repeat in the second half.

The international picture is structural rather than one-off. As Mounjaro launches broaden into markets such as China — where inclusion on the National Reimbursement Drug List drove 93% constant-currency growth — the mix moves toward lower-priced geographies. Volume more than compensates today. It is a narrower margin of safety than the headline growth rate implies, and it is the single most important thing to monitor in this name.

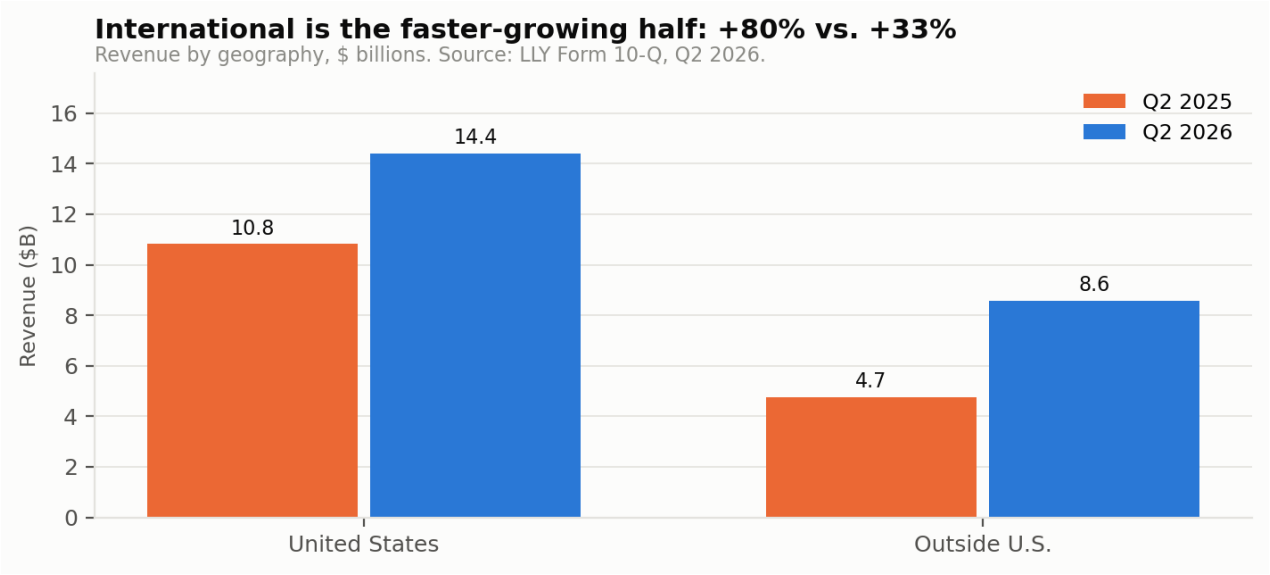


Figure 4 — Revenue by geography, Q2 2026 versus Q2 2025.

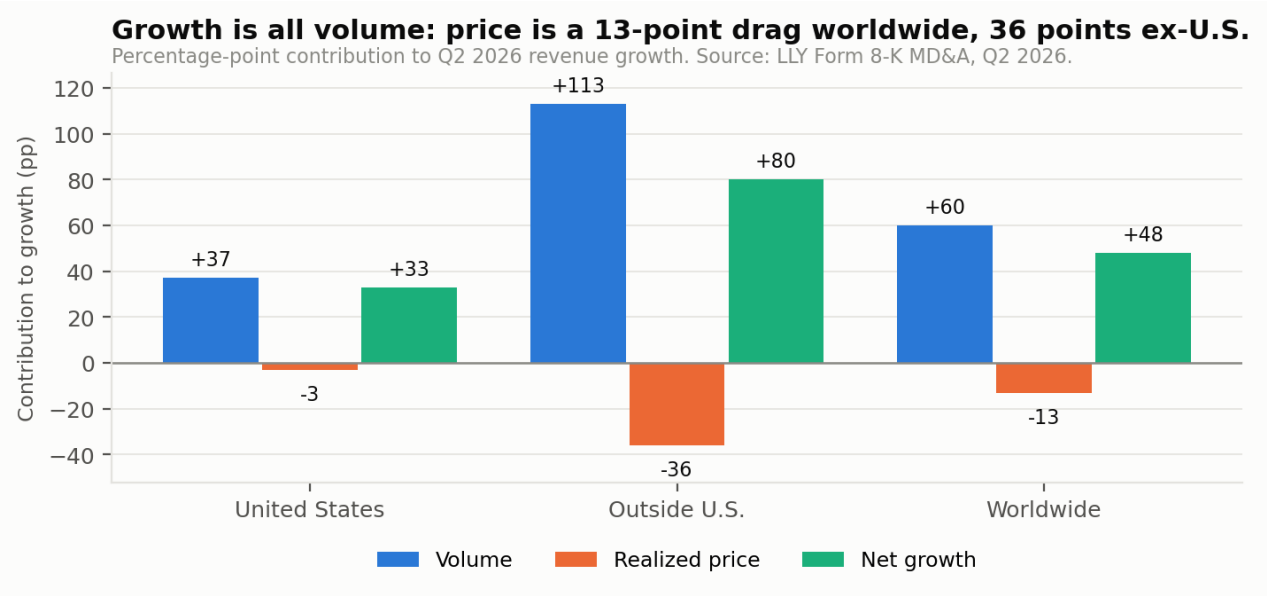


Figure 5 — Decomposition of Q2 2026 revenue growth into volume and realised price.

2. Margins, IPR&D, and what the GAAP numbers hide

Non-GAAP gross margin expanded 130 basis points to 86.3%, which management attributed to favourable product mix and improved production costs. Operating expense grew more slowly than revenue in both categories: R&D rose 14.5% to $3,819m (16.6% of revenue, down from 21.4%), and marketing, selling and administrative rose 24.6% to $3,430m (14.9% of revenue, down from 17.7%). The company-defined performance margin consequently jumped 900 basis points to 54.8%.

GAAP operating margin moved the other way, falling from 44.1% to 39.1%. The entire difference is below the performance-margin line: $2,776m of acquired IPR&D (versus $154m a year ago) from the completed acquisitions of Orna Therapeutics, Ajax Therapeutics, Centessa Pharmaceuticals and Kelonia Therapeutics, plus $703m of asset impairment, restructuring and other special charges against nil a year ago.

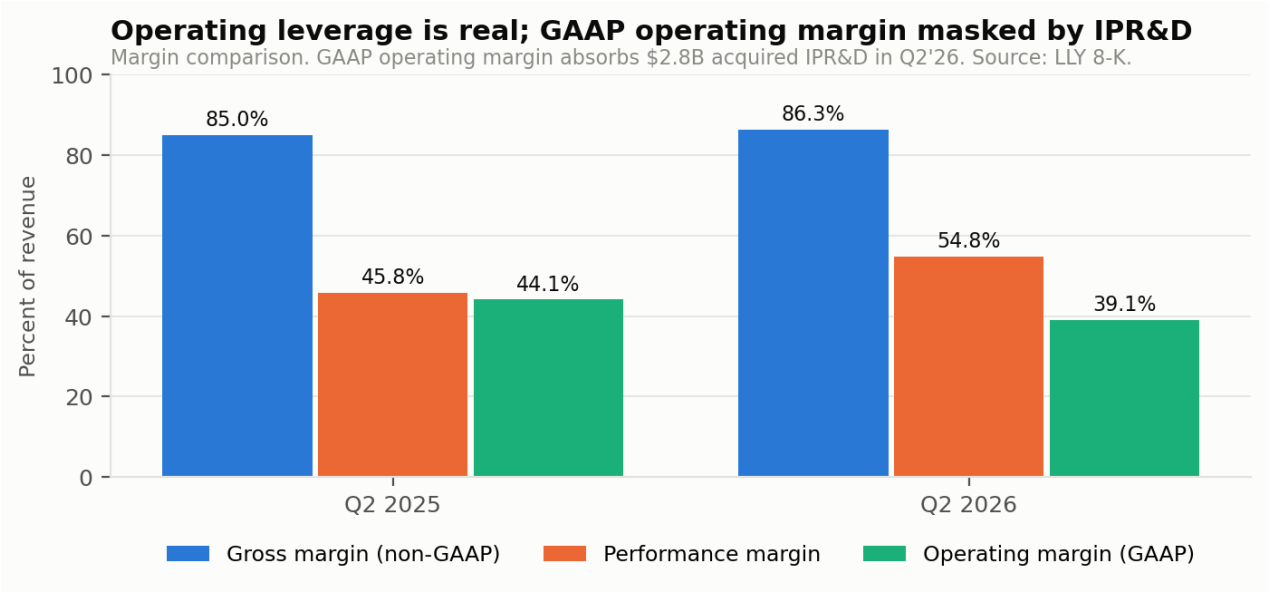


Figure 6 — Margin comparison, Q2 2025 versus Q2 2026.

The EPS 'miss' that was not one

Reported sources disagree on whether Lilly beat or missed on earnings, and both are working from defensible consensus figures. Against a consensus of roughly $6.01 — a number compiled before the quarter's business development was known — the $8.38 non-GAAP result was a 39% beat. Against an IPR&D-inclusive consensus of $8.84, it was a 5.2% miss.

Neither comparison is informative on its own. Acquired IPR&D is a real cash outflow — $3,486m of IPR&D purchases and $9,805m of cash paid for acquisitions in the first half — but it is discretionary, lumpy, and by construction unforecastable by analysts who do not know which deals will close in a given quarter. Lilly's own guidance excludes IPR&D incurred after 30 June 2026 for exactly this reason. The operating result, excluding the charge, was $11.41 per share against $6.31 a year ago.

The practical reading: revenue was a clean and large beat, operating leverage was real, and the EPS line is a bookkeeping question about how to treat deal spending. The market took this view — shares closed higher on the day of the release.

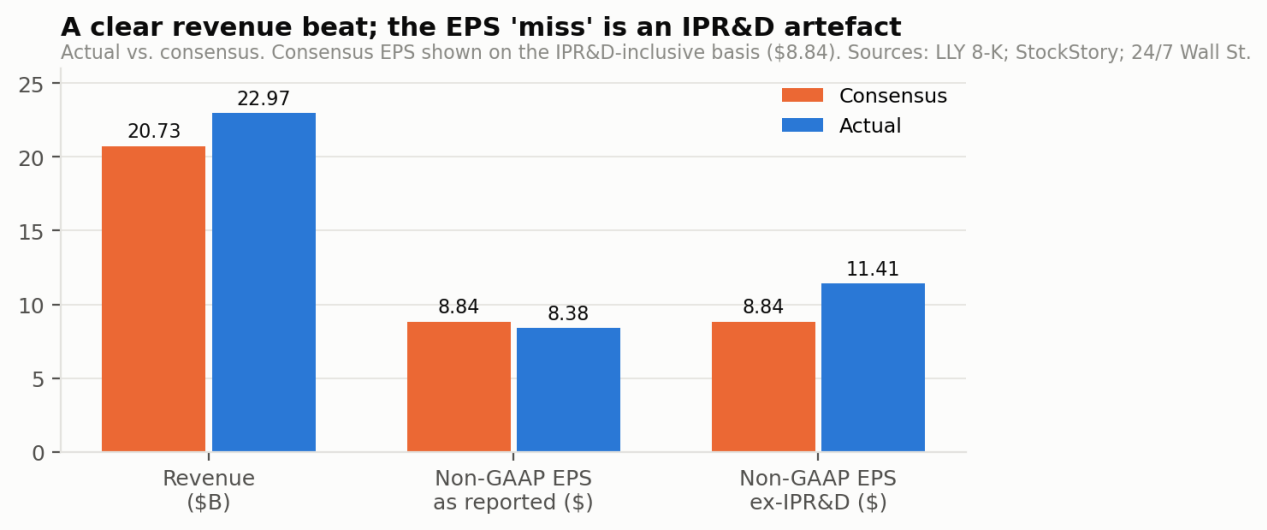


Figure 7 — Actual versus consensus on the three relevant bases.

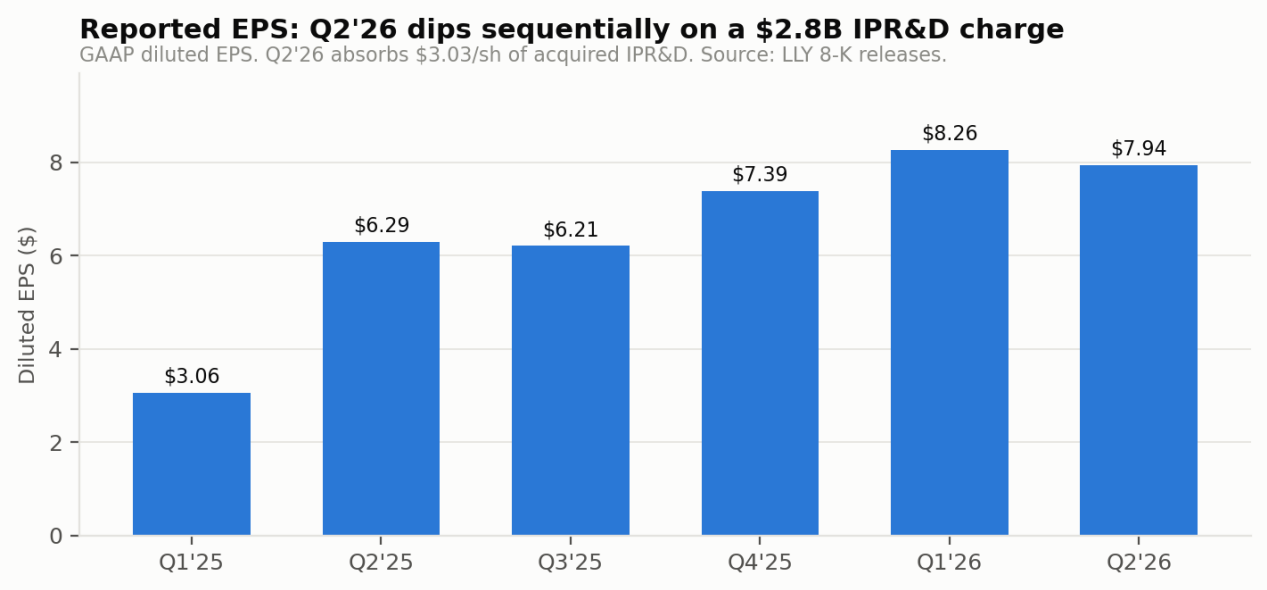


Figure 8 — Reported GAAP diluted EPS by quarter. The Q2 2026 sequential decline is entirely the acquired IPR&D charge.

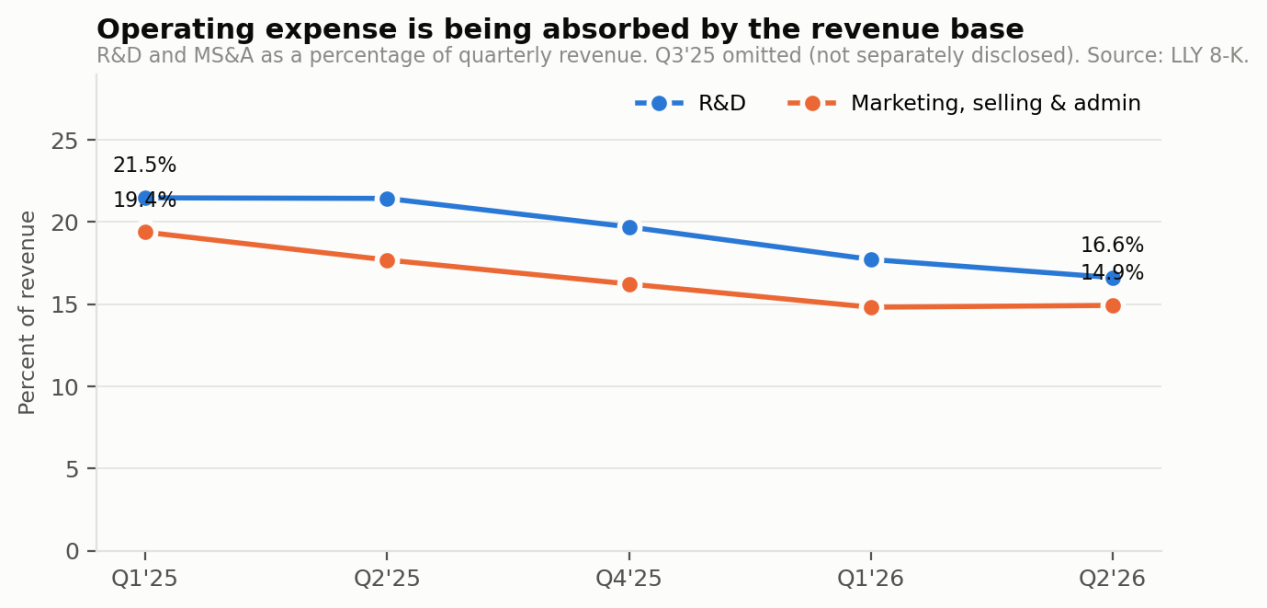


Figure 9 — R&D and MS&A as a percentage of revenue. Q3 2025 omitted (not separately disclosed).

3. Guidance

Metric	Prior guidance	Updated guidance	Direction
Revenue	**$82 – $85 billion**	**$85 – $87 billion**	**Raised $2.5bn at midpoint**
Performance margin	47.0% – 48.5%	49.0% – 50.5%	Raised 200 bps at midpoint
Tax rate	18% – 19%	18% – 19%	Unchanged
EPS, non-GAAP	**$35.50 – $37.00**	**$35.50 – $36.50**	**Midpoint down $0.25; range narrowed**

Source: Eli Lilly Form 8-K, 5 August 2026. Guidance excludes acquired IPR&D incurred after 30 June 2026 and assumes approximately 894 million diluted shares with FX of 1.14 (EUR), 153 (JPY) and 7.1 (CNY).

The EPS guidance requires unpacking. Underlying non-GAAP EPS was raised $2.78 at the midpoint on the strength of first-half performance. That raise was more than offset by the $3.03 of acquired IPR&D already booked in Q2, producing a midpoint that moves down $0.25 while the underlying business outlook improved materially. The narrowing of the top end from $37.00 to $36.50 is arithmetic, not a signal of deteriorating confidence.

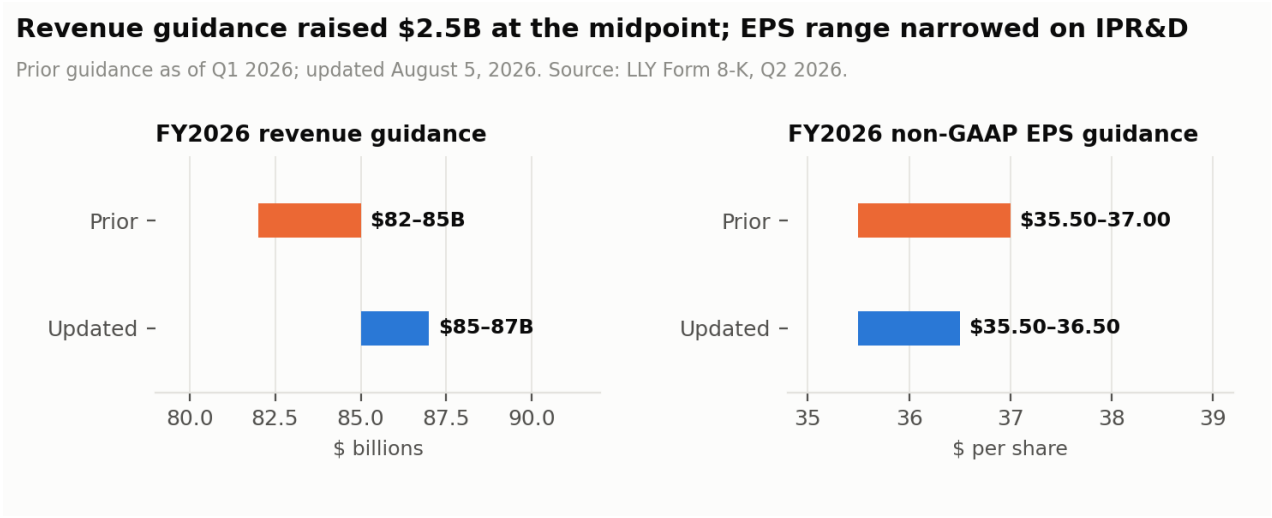


Figure 10 — FY2026 guidance, prior versus updated.

What the second-half implied guidance says

Analyst Akash Tewari of Jefferies pressed management on an apparent deceleration: the raised guidance implies second-half revenue growth below the 48% posted in Q2, which sits oddly beside a Medicare access expansion and an oral GLP-1 that is only beginning to ramp. Montarce gave three reasons. First, the one-time rebate-estimate benefits in Q2 do not repeat. Second, the second half of 2025 contained a bolus of Mounjaro launches in new countries; future growth must come from penetration within existing markets rather than new market entry. Third, normal seasonality — European softness in Q3, US type-2 diabetes seasonality in Q4 — is embedded.

He added that dollar growth, as opposed to percentage growth, is expected to continue significantly in the second half. At the guidance midpoint, implied H2 revenue is approximately $43.2bn against $42.8bn in H1 — modest sequential progression that leaves room for a further raise if the Medicare Bridge ramp and Foundayo inflection hold. The guidance looks conservative rather than cautionary.

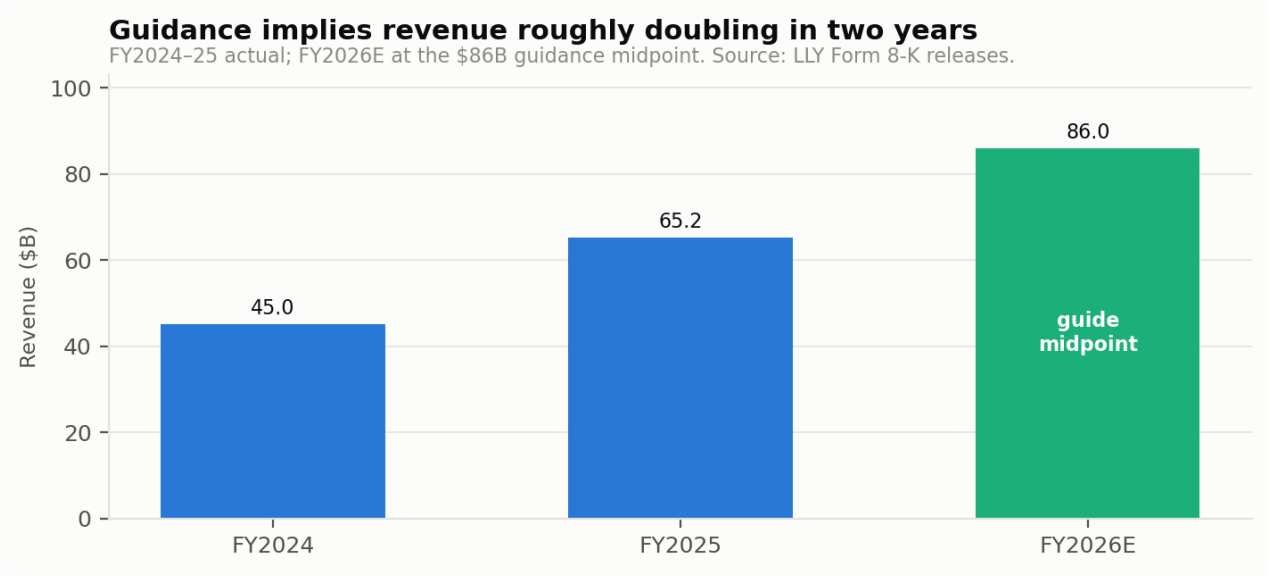


Figure 11 — Annual revenue, FY2024 to FY2026 guidance midpoint.

4. Pipeline, launches and catalysts

Orforglipron / Foundayo — the oral GLP-1

Foundayo, approved by the FDA on 1 April 2026 for chronic weight management, contributed $98m in its first substantial quarter. Management was candid that the launch has been slower than hoped. Commercial access across all three major PBMs was only secured in June; direct-to-consumer marketing launched in early July. The prescriber base expanded from 8,000 to 36,000, and Patrik Jonsson and Ilya Yuffa both pointed to a late-July inflection in which weekly volume nearly doubled against a month earlier, with new patient starts approaching one in four on Foundayo.

A sampling programme covering the first 30 days of the initial dose is running, mirroring conventional retail small-molecule promotion. This will dampen reported IQVIA scripts relative to true patient starts, so early prescription data should be read with care. Foundayo's principal differentiator against Novo Nordisk's oral Wegovy is the absence of food restrictions. A US submission for type 2 diabetes has been completed on the back of the ACHIEVE programme, in which ACHIEVE-3 showed superiority to oral semaglutide at both 7mg and 14mg on glycaemic control and weight loss.

Retatrutide — the next franchise, with a regulatory complication

Phase 3 results across TRIUMPH-1 (obesity), TRIUMPH-2 (diabetes with obesity) and TRIUMPH-3 (severe obesity with cardiovascular disease) were positive, with weight loss that Ricks described as approaching bariatric-surgery levels at the highest doses. The clinical package is complete for obesity, obstructive sleep apnoea and knee osteoarthritis pain. A roughly 75% reduction in osteoarthritis pain was cited from TRIUMPH-4 and TRIUMPH-1. Phase 3 work continues in MASH, where the glucagon mechanism may confer a distinct advantage, and a chronic lower back pain study reads out next year.

The complication is the filing pathway. Lilly maintains retatrutide qualifies as a biologic under the amino-acid-count rule; a prior court decision went the other way and litigation is active. Ricks said the company is seeking FDA agreement on a BLA pathway, that CMC data are still being assembled, and that investors will be updated on filing status. The stated target is a Q1 2027 US BLA submission. This is a genuine, dateable risk to the timeline rather than a theoretical one, and it deserves a place on any catalyst calendar.

The rest of the pipeline

- Insulin efsitora alfa: CHMP positive opinion for EU approval in type 2 diabetes.
- Verve-102 (PCSK9 gene editing): Phase 1b showed an 88% PCSK9 reduction and 62% LDL reduction at the highest dose from a single infusion.
- Jaypirca: +56% to $192m; EU approval in CLL across all lines. BRUIN CLL-322 showed a 45% reduction in risk of progression or death when added to venetoclax/rituximab.
- Selpercatinib: LIBRETTO-432 showed an 83% reduction in recurrence or death in adjuvant RET fusion-positive NSCLC (ASCO 2026).
- Inluriyo (imlunestrant): >50% share of new prescriptions in the metastatic breast cancer oral SERD market after two quarters; $75m in the quarter. EMBER-4 adjuvant data expected.
- Ebglyss: +131% to $201m; FDA approval for every-eight-week maintenance dosing in atopic dermatitis; Almirall partnership exploring 12-week dosing with data in 2027.
- Kisunla: $167m against $49m. P-tau217 blood testing volume more than doubled year on year, which is the rate-limiting step for the whole anti-amyloid category.

- New therapeutic areas via M&A: AtaiBeckley (treatment-resistant depression, BPL-003), Centessa (cleminorexton, orexin-2 agonist for central hypersomnias), Curevo and LimmaTech and Vaccine Company (vaccines), Ajax (AJ1-11095, type II JAK inhibitor), Kelonia (KLN-1010, in vivo CAR-T).

Chief Scientific Officer Dan Skovronsky's summary is worth recording verbatim in substance: if you eliminated the incretin programmes, what remains would still be a strong pipeline. That is the bull case for the durability of the franchise beyond tirzepatide, and the four acquisitions closed in the quarter are the company putting capital behind it.

5. Access, pricing and policy

The Medicare GLP-1 Bridge programme

Launched 1 July 2026, the programme gives approximately 20 million eligible Americans a $50 monthly out-of-pocket cost for obesity medicines and expands overall access to obesity medication by roughly 35%. Early data described on the call: 60–70% of enrollees were new to obesity treatment, and around 80% chose injectables over orals. LillyDirect has been enabled to fulfil Bridge requirements.

This is the most important commercial development of the quarter and the least visible in the Q2 numbers, because it began the day after the quarter closed. It converts a previously uncovered population into a reimbursed one at a price point well below commercial net price.

Direct-to-consumer and net price

Self-pay accounted for 45% of total Zepbound prescriptions and 55% of new prescriptions in the quarter, at LillyDirect prices ranging from $299 to $449 per month. Umer Raffat of Evercore pressed on Zepbound net price of roughly $580 per prescription against Wegovy at roughly $380. Montarce's answer identified two drivers: differences in script size across channels, and medical exceptions that arose because CVS formulary restrictions pushed patients into the cash market or to competitors. He characterised the medical-exception effect as short-term, and confirmed that CVS formulary access from Q4 2026 will bring net price down — offset in guidance by volume.

Investors should expect reported net price per prescription to fall in Q4 and into 2027. Management has told them so. The question is whether volume scales fast enough to keep dollar growth intact, and the Bridge programme is the main reason to think it will.

Tariffs and most-favoured-nation pricing

Lilly agreed a drug-pricing arrangement with the US administration under which Medicare covers Zepbound and Mounjaro at $245 from 1 April 2026 — a substantial discount to prior net prices — in exchange for a three-year tariff exemption. That removes a tail risk that weighed on the sector through 2025 and converts it into a known, quantified price concession. Management offered no new commentary on most-favoured-nation pricing on the Q2 call; the policy remains an open variable for the sector rather than a resolved one.

Competition

Novo Nordisk's oral Wegovy is the direct competitive threat to Foundayo. Jonsson noted that Lilly's combined incretin market share held steady despite Novo's entry, and that orals appear to be expanding the incretin market rather than purely dividing it. On generic semaglutide, he pointed to India and Brazil, where Mounjaro total prescriptions continued to grow after generic launch on the strength of head-to-head superiority data, and to Canada, where generic entrants have faced significant supply constraints. Lilly is reported to hold approximately 60% of the weight-loss drug market.

6. Balance sheet, cash flow and capital allocation

$ millions	30 Jun 2026	31 Dec 2025	Change
Cash and cash equivalents	**8,950**	**7,268**	**+1,682**
Accounts receivable	20,083	17,760	+2,323
Inventories	16,793	13,744	+3,049
Goodwill	8,849	5,898	+2,951
Other intangibles, net	18,110	6,521	+11,589
Property and equipment, net	29,286	24,675	+4,611
Total assets	**142,283**	**112,476**	**+29,807**
Short-term borrowings and current LTD	7,050	1,635	+5,415
Sales rebates and discounts	21,122	17,382	+3,740
Long-term debt	47,858	40,868	+6,990
Total equity	**33,879**	**26,535**	**+7,344**

Source: Eli Lilly Form 10-Q, condensed consolidated balance sheet.

Three items deserve attention. Intangibles nearly tripled and goodwill rose 50%, both direct consequences of the four acquisitions. Total debt rose to $54.9bn, with short-term borrowings including $5.3bn of commercial paper that did not exist at year end — this is how the acquisitions were funded, alongside $8.9bn of long-term issuance. The sales rebates and discounts liability of $21.1bn, up $3.7bn, is the balance-sheet expression of the gross-to-net gap and is worth tracking as a leading indicator of realised price.

Inventories of $16.8bn against quarterly cost of sales of $3.3bn represent roughly five quarters of cover. For a company that spent 2024 supply-constrained and is launching two new molecules, building inventory ahead of demand is the correct decision; it is not a demand warning.

Cash flow

First-half operating cash flow of $16,023m was more than triple the $4,753m of the prior year — a swing driven by higher net income and a far smaller working-capital drag ($2,334m versus $5,627m). Capital expenditure of $5,259m (up 64%) funds the manufacturing build-out. Free cash flow was approximately $10,764m.

Deployment was aggressive: $9,805m of cash for acquisitions, $3,486m for IPR&D purchases, $3,094m of dividends and $3,957m of buybacks. Total outflow across those four lines was $20.3bn against $10.8bn of free cash flow, with the gap funded by debt. This is a company deliberately levering a cash-flow windfall into pipeline and capacity. Whether that is the right trade depends entirely on the acquired assets, and it will not be testable for years.

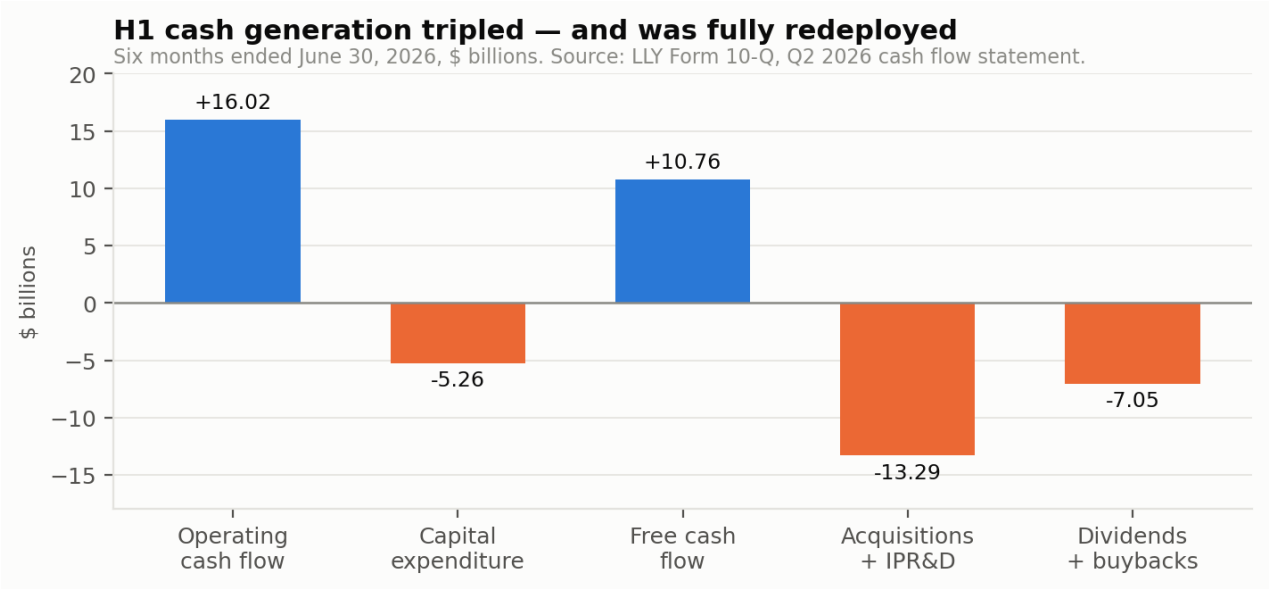


Figure 12 — First-half cash generation and deployment.

Manufacturing and supply chain

Lilly committed an additional $4.5bn to its Indiana sites in May 2026, bringing cumulative Indiana investment since 2020 to approximately $21bn. The Lebanon campus opened its first dedicated genetic-medicine facility (Lilly Lebanon Advanced Therapies) in May; the adjacent API site is scheduled to open in 2027 and is described as the largest API production site in US history, with planned output covering tirzepatide for Mounjaro and Zepbound plus capacity for Foundayo and investigational retatrutide. The Limerick, Ireland site — a near-$1bn investment and the company's most technically advanced monoclonal antibody facility — produced its first commercial batch in the quarter.

Supply is no longer the binding constraint: Jonsson stated explicitly that supply is not limiting international launches, and the sampling programme for Foundayo is only possible because supply is ample. The strategic point is that domestic API capacity is also the company's structural answer to tariff exposure, which is why the three-year exemption and the Indiana build-out should be read as a single policy posture rather than two separate stories.

7. Ownership, short interest and positioning

Metric	Value	As of / source
Share price	**$1,146.41**	**18 Sep 2026, StockAnalysis.com**
Market capitalisation	$1.02 trillion	18 Sep 2026
Enterprise value	$1.07 trillion	18 Sep 2026
Shares outstanding	891.4 million	18 Sep 2026
Institutional ownership	89.5%	Latest 13F aggregation
Insider ownership	0.17%	Latest Forms 3/4/5
Shares short	7.89 million	FINRA, 31 Aug 2026 settlement
Short interest as % of float	0.89%	FINRA / StockAnalysis.com
Days to cover	2.76	FINRA reported ADV
Change in short interest	+1.33 million shares	vs. prior settlement
Beta	0.50	StockAnalysis.com
52-week price change	+51.0%	18 Sep 2026
50-day / 200-day moving average	$1,177 / $1,066	18 Sep 2026

Short interest at 0.89% of float is negligible for a company of this size and is not a meaningful source of either support or pressure. The 1.33 million share increase into the 31 August settlement is worth noting as a directional tell — some incremental scepticism after the print — but on this base it is noise rather than signal. Days to cover of 2.76 means no squeeze dynamic exists.

Largest holders

Holder	Shares (m)	% outstanding	As of
Lilly Endowment Inc.	**91.9**	**~10.3%**	**Q1 2026 13F**
The Vanguard Group	82.0	9.14%	Q1 2026 13F
BlackRock, Inc.	67.5	7.55%	Q1 2026 13F
State Street Corporation	n/d	n/d	Q1 2026 13F
PNC Financial Services Group	n/d	n/d	Q1 2026 13F
FMR LLC (Fidelity)	n/d	n/d	Q1 2026 13F
Capital Research Global Investors	n/d	n/d	Q1 2026 13F
Geode Capital Management	n/d	n/d	Q1 2026 13F
Morgan Stanley	n/d	n/d	Q1 2026 13F
Capital World Investors	n/d	n/d	Q1 2026 13F

Source: aggregated Form 13F filings for the quarter ended 31 March 2026. Share counts for holders below the top three were not verifiable at the level of precision required for publication and are marked n/d. Note the unusual register: Lilly Endowment Inc., a private philanthropic foundation, is the largest single holder — a stabilising feature of the shareholder base that most mega-caps do not have.

Sell-side positioning

Of approximately 45 covering analysts, 73% carry a Buy or Strong Buy rating, with 9 Holds and 3 Sells. The average 12-month price target sits in the $1,316–$1,325 range, implying roughly 15–16% upside from the 18 September price, with a high of $1,600 and a low of $930. Berenberg upgraded from Hold to Buy on 15 September, raising its target from $1,220 to $1,400. The average target has been revised up roughly 4% over three months.

The distribution matters more than the average. A $930-to-$1,600 spread on a $1.02 trillion company reflects genuine disagreement about the durability of incretin pricing and the terminal size of the obesity market, not merely different discount rates.

8. Valuation

Multiple	Value	Basis
P/E, trailing twelve months	**38.7x**	**TTM EPS $29.79**
P/E, FY2026 guidance midpoint	**31.8x**	**Non-GAAP EPS $36.00**
P/E, FY2026 midpoint ex-Q2 IPR&D	**29.4x**	**Non-GAAP EPS $39.03**
P/E, forward (NTM)	28.3x	Consensus NTM
EV/EBITDA	25.6x	TTM
Price / sales	12.9x	TTM revenue $79.7bn
Price / book	30.3x	Equity $33.9bn
PEG	1.32	StockAnalysis.com
Free cash flow yield	1.78%	TTM FCF $18.2bn
Dividend yield	0.60%	DPS $6.92, payout 22.5%

Calculated at $1,146.41 per share. Ratios other than the guidance-based lines are from StockAnalysis.com as of 18 September 2026.

The stock trades at 31.8x the midpoint of management's own FY2026 guidance, or 29.4x if the Q2 IPR&D charge already absorbed is added back. Against a business that grew revenue 48% and underlying EPS 81% in the quarter, and that guides to roughly 32% full-year revenue growth, the multiple is not demanding on near-term numbers. The price embeds a view about years three through ten, not about 2026.

Scenario framework

The following scenarios are an illustrative framework for sizing outcomes, not a price target or a recommendation. They hold the FY2026 guidance midpoint as the anchor and vary the two assumptions that actually move the valuation: the durability of net price and the success of the post-tirzepatide pipeline.

Scenario	Key assumptions	FY2028E EPS	Multiple	Implied value
Bear	**Net price declines accelerate past 10%/yr; Foundayo undershoots against oral Wegovy; retatrutide BLA slips past 2027 on the biologic pathway dispute**	**$42**	**20x**	**~$840**
Base	Volume growth offsets high-single-digit price erosion; Bridge programme delivers; Foundayo ramps through 2027; retatrutide files Q1 2027	$52	25x	~$1,300
Bull	Obesity market compounds past current penetration; retatrutide extends into MASH, OSA and osteoarthritis; non-incretin pipeline contributes materially	$60	30x	~$1,800

Illustrative only. EPS figures are the author's scenario assumptions, not company guidance and not consensus. Sell-side targets currently span $930 to $1,600, a range broadly consistent with this framework.

9. What to watch

- Q4 2026 net price per prescription once CVS formulary access takes effect — management has pre-announced a decline; the question is the offset from volume.
- Medicare GLP-1 Bridge enrolment and the injectable/oral split, which began 1 July and is entirely absent from Q2 results.
- Foundayo weekly prescription trend, adjusted for the sampling programme's distortion of IQVIA data.

- Resolution of the retatrutide BLA-versus-NDA pathway and the associated litigation; a Q1 2027 submission target depends on it.
- Gross-to-net: the sales rebates and discounts liability, now $21.1bn, as a leading indicator of realised price.
- Ex-US price mix as Mounjaro penetrates lower-priced reimbursement systems.
- EMBER-4 adjuvant readout for imlunestrant and the Verzenio erosion curve in oncology.
- Any revival of most-favoured-nation pricing policy, which the three-year tariff exemption does not address.

10. Risks

Price is the dominant risk and it is already materialising: a 13% worldwide realised-price decline, 9% in the US excluding one-time rebate revisions, and 36% outside the US. Volume is outrunning it comfortably today. If obesity-market volume growth normalises before price erosion does, the revenue algorithm inverts quickly.

Concentration is the second risk. Two tirzepatide brands are 65% of revenue and 85% of growth. Any safety signal, manufacturing interruption, formulary exclusion or competitive-efficacy surprise in that molecule is not diversifiable within the P&L.

Execution risk on capital deployment is third. $13.3bn of first-half acquisition and IPR&D spend, funded substantially with debt, converts balance-sheet strength into pipeline optionality. The four Q2 acquisitions span oncology, immunology, psychiatry and vaccines — areas where Lilly has less commercial history than in cardiometabolic health.

Policy risk is reduced but not eliminated. The tariff exemption runs three years. Most-favoured-nation pricing remains unresolved, and the Medicare arrangement itself establishes a $245 reference price that will be difficult to move upward.

Finally, the retatrutide filing pathway is a discrete, dateable legal risk. An adverse outcome does not impair the clinical data but could delay a launch that a meaningful portion of the valuation depends on.

Sources

Every figure in this report is traceable to one of the following. Primary filings were downloaded from SEC EDGAR and are archived alongside this document in 01_filings.

Primary — company filings and disclosures

- Form 8-K, Q2 2026 earnings release, furnished 5 August 2026 — https://www.sec.gov/Archives/edgar/data/0000059478/000005947826000077/q226lillysalesandearningsp.htm
- Form 10-Q, quarter ended 30 June 2026, filed 5 August 2026 — https://www.sec.gov/Archives/edgar/data/59478/000005947826000081/lly-20260630.htm
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- Eli Lilly Q2 2026 earnings call transcript (Seeking Alpha) — https://seekingalpha.com/article/4930902-eli-lilly-and-company-lly-q2-2026-earnings-call-transcript
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Market, ownership, short interest and consensus

- Eli Lilly statistics and valuation (StockAnalysis.com) — price, multiples, short interest, ownership — https://stockanalysis.com/stocks/lly/statistics/
- Eli Lilly institutional holders, Form 13F aggregation (Fintel) — https://fintel.io/so/us/lly
- Eli Lilly short interest (Fintel) — https://fintel.io/ss/us/lly
- Eli Lilly Q2 2026 earnings coverage (CNBC) — https://www.cnbc.com/2026/08/05/eli-lilly-lly-earnings-q2-2026.html
- Eli Lilly beats expectations in Q2 CY2026 (StockStory via FinancialContent) — consensus figures — https://markets.financialcontent.com/stocks/article/stockstory-2026-8-5-eli-lilly-nyselly-beats-expectations-in-strong-q2-cy2026-full-year-outlook-slightly-exceeds-expectations
- Eli Lilly earnings history and consensus (24/7 Wall St.) — https://247wallst.com/companies/lly/earnings/

Pipeline, manufacturing and policy

- FDA approves Lilly obesity pill, triggering battle with Novo Nordisk (BioPharma Dive) — https://www.biopharmadive.com/news/fda-approves-lilly-obesity-pill-orforglipron-foundayo-novo/816359/
- Eli Lilly's obesity pill approved by FDA (STAT News) — https://www.statnews.com/2026/04/01/eli-lilly-obesity-pill-approved-orforglipron-foundayo/
- Eli Lilly will file for approval of retatrutide obesity drug in 2027 (CNBC) — https://www.cnbc.com/2026/07/23/eli-lilly-will-file-for-approval-of-retatrutide-obesity-drug-in-2027.html
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- Inside Lilly's manufacturing expansion (Eli Lilly) — https://www.lilly.com/news/stories/new-manufacturing-facilities

- Eli Lilly statement on drug pricing, innovation and tariffs (Pharmaceutical Executive) — https://www.pharmexec.com/view/eli-lilly-statement-drug-pricing-innovation-tariffs

Notes on data quality. Q3 2025 revenue and earnings are derived as FY2025 less the three separately reported quarters, because Lilly does not restate a standalone Q3 in the Q2 2026 release. Consensus estimates are third-party compilations and differ across vendors, materially so on EPS for this quarter because of the treatment of acquired IPR&D; both figures encountered are reported in section 2 rather than one being chosen. Institutional holdings reflect Form 13F filings for the quarter ended 31 March 2026 and are therefore up to five months stale. Short interest is FINRA data for the 31 August 2026 settlement date. Market data is as of 18 September 2026.

This document was prepared for internal research purposes from public sources. It is not investment advice, is not personalised to any investor's circumstances, and is not a recommendation to buy or sell any security. The scenario analysis in section 8 is illustrative. No rating or price target is issued. Figures should be verified against the primary filings before being relied upon.